

Durvalumab (Imfinzi®) with cisplatin and gemcitabine

Durvalumab (also known as Imfinzi[®]) with cisplatin and gemcitabine is a cancer drug treatment. It is used to treat bile duct cancer (cholangiocarcinoma) and muscle-invasive bladder cancer (MIBC).

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What is durvalumab with cisplatin and gemcitabine?

Durvalumab with cisplatin and gemcitabine is a combination of cancer drugs. It is used for:

[Bile duct cancer \(cholangiocarcinoma\)](#) that:

- cannot be removed with surgery (unresectable)
- has [spread](#) into other tissues or lymph nodes nearby or to another part of the body, such as the lungs.

[Muscle-invasive bladder cancer \(MIBC\)](#)

- given before surgery to remove the bladder, followed by treatment with durvalumab alone after surgery.

Durvalumab with cisplatin and gemcitabine is a combination of the following drugs:

- [durvalumab](#) – an immunotherapy drug (also called a checkpoint inhibitor)
- [cisplatin](#) – a chemotherapy drug
- [gemcitabine](#) – a chemotherapy drug.

It is best to read this information with our general information about [immunotherapy](#), [chemotherapy](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How durvalumab with cisplatin and gemcitabine is given

You usually have durvalumab with cisplatin and gemcitabine in a chemotherapy day unit or clinic as an outpatient. Sometimes you may have this treatment during a stay in hospital.

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Before or on the day of each treatment, you will have a blood test. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your cancer drugs.

Your nurse will usually give you anti-sickness drugs before the cancer drugs. You will have your cancer treatment through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in the arm or hand
- a [central line](#) – a fine tube that goes under the skin of the chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in the arm and goes up into a vein in the chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on the chest or arm and goes into a vein in the chest.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Your course of treatment

Treatment for bile duct cancer

Durvalumab with cisplatin and gemcitabine is usually given in 2 phases. Your cancer team will discuss your treatment plan with you.

Combination phase

You usually have up to 8 cycles of combination treatment. Each cycle lasts 3 weeks. In each cycle, you have treatment on day 1 and day 8 and no treatment on the other days.

On day 1 of each cycle, you have:

- durvalumab as a drip (infusion) into a vein – this is given over 60 minutes (1 hour)
- gemcitabine as a drip over 30 minutes
- cisplatin as a drip over 60 minutes.

On day 8 of each cycle, you have:

- gemcitabine as a drip over 30 minutes
- cisplatin as a drip over 60 minutes.

Cisplatin can affect the kidneys. You will be given extra fluids through the drip before and after the cisplatin. You may also have a drug called mannitol, which is given as a drip. Or you will have a drug called furosemide, which is given as a tablet or an injection. This is to prevent kidney damage.

Your nurse will explain how long it will take to have the extra fluids. They will also give you advice about how much fluid to drink in the 24 hours after your treatment.

After you have had up to 8 cycles of the combination phase, you may move on to the maintenance phase.

Maintenance phase

In this phase, you have durvalumab every 4 weeks. You have it for as long as it is working for you and you are able to cope with side effects. You have it as a drip into a vein over 60 minutes.

Not everyone has the maintenance phase. Your cancer doctor will discuss whether this is suitable for you.

Treatment for bladder cancer

You have durvalumab with cisplatin and gemcitabine:

- before surgery - every 21 days (3 weeks) for up to 4 cycles
- after surgery – you have durvalumab on its own once every 28 days.

You usually continue with durvalumab for up to 8 cycles for as long as it is working and the side effects can be managed.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Delayed side effects

This treatment may also cause delayed side effects. These can start weeks, months, or sometimes more than a year after you finish treatment.

If you have symptoms or side effects that start during treatment or after treatment ends, contact the hospital straight away on the 24-hour number. Side effects can become serious if they are not treated.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Your patient alert card

They will also give you a card with information about your treatment. It is important to keep the card with you during treatment and after treatment ends. You should show it to any doctor or healthcare professional caring for you.

Side effects while treatment is being given

Some people have side effects while they are having a cancer drug or shortly after they have it.

Allergic reaction

Some people have an allergic reaction while having this treatment. Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy

- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your treatment. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

The drug leaks outside the vein

Sometimes cancer drugs that are given into a vein may leak outside the vein. If this happens, some drugs can damage the tissue near the vein. This is called extravasation. Extravasation is not common, but it is important that it is dealt with quickly. If you have any of the following symptoms around the vein during or after your treatment, tell your nurse straight away:

- stinging
- pain
- swelling
- if you have white skin, the area may become red
- if you have black or brown skin, the area may become darker.

Immune system side effects

Durvalumab is an immunotherapy drug, so it can make the immune system too active. This can cause the following side effects. Some of them are rare. But it is important to know about them.

Do not try to treat these side effects yourself. If you have these side effects during treatment or after it ends, always contact the hospital straight away on the 24-hour number.

Diarrhoea or tummy pain

You may have [diarrhoea](#) or tummy (abdominal) pain. Sometimes it can cause serious inflammation of the bowel called colitis.

Contact the hospital straight away on the 24-hour number if you have any of the following symptoms during treatment, or after treatment ends:

- passing more stools (poo) than is normal for you
- passing stools at night
- if you have a stoma, it is more active or needs emptied more often than usual
- watery or loose stools
- uncomfortable tummy cramps or pain
- a temperature over 37.5°C

- blood or mucus (slime) in your stool.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Skin changes

This treatment may cause inflammation in the skin. This is usually mild, but if it is not treated it can become serious. You may need creams, steroids or other medicines to treat skin problems.

Contact the hospital on the 24-hour number as soon as possible, if you notice any skin changes during treatment, or after treatment ends. This includes:

- a rash or bumps on the skin
- dry, tight or itchy skin
- areas that are a different colour to your usual skin tone.

Here are some things you can do to take care of your skin during treatment:

- Wash with lukewarm water and mild, unperfumed, soap-free cleansers.
- Moisturise your skin regularly with unperfumed moisturisers for dry skin conditions.
- Use sun cream of at least SPF 30 to protect your skin from the sun. Cover up with clothing and a hat.

Contact the hospital straight away on the 24-hour number if you have any of these serious skin symptoms:

- a rash that is spreading
- swollen, blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain.

Hormone changes

This treatment may permanently affect glands in the body that produce hormones. Hormones control different processes in the body. You will have regular blood tests to check some hormone levels, such as your thyroid. You may need to take tablets daily to correct your hormone levels.

Contact the hospital on the 24-hour number if you have any of these side effects during treatment or after it ends:

- increased sweating
- problems with constipation
- weight gain or loss
- dizziness or fainting
- feeling or being sick
- feeling more hungry or thirsty than usual
- loss of sex drive
- passing urine (peeing) more often than usual
- headaches that do not go away
- feeling more tired than usual (fatigue)
- mood changes
- changes to your heartbeat (palpitations).

Muscle and joint pain

This treatment can cause sore or weak muscles. Your joints may also be stiff, sore or swollen. This is called arthritis. If you already have problems with joint pain, this may become worse.

Contact the hospital on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- new or worsening aches or pains
- new or worsening stiffness in a joint
- swollen joints or muscle weakness.

Effects on the liver

This treatment may affect how your liver works. It can cause liver inflammation called hepatitis. You will have regular blood tests to check how your liver is working. Tell your doctor, nurse or pharmacist if you have loss of appetite.

Sometimes liver changes can be serious. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- yellow skin or eyes
- feeling very sleepy
- dark urine (pee)

- unexplained bleeding or bruising
- pain in the right side of your tummy (abdomen).

If you have black or brown skin, changes in your skin colour may be less noticeable. You might first notice the whites of your eyes look yellow, or that the colour of your urine has changed.

Effects on the kidneys

This treatment can affect how the kidneys work. It may cause kidney inflammation called nephritis. You will have regular blood tests to check how your kidneys are working.

Contact the hospital straight away on the 24-hour number during treatment, or after it ends, if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Effects on the nervous system

Rarely, this treatment can affect the brain and nerves. These effects may be mild at first but can become serious if not treated. Symptoms may include:

- weakness, numbness or tingling in your arms, legs or face
- a headache that does not get better
- seeing double or having drooping eyelids
- difficulty turning or bending your neck
- drowsiness or confusion
- problems with moving or speaking
- fits (seizures).

If you have any of these symptoms during treatment or after it ends, it is important to either:

- contact the hospital straight away on the 24-hour number
- go to the hospital straight away
- call **999**.

Do not drive yourself to hospital.

Effects on the heart

This treatment can affect [how the heart works](#). You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)

- swollen feet and ankles.
- flu-like symptoms, such as body aches, pains and tiredness.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Raised blood sugar levels

This treatment may raise your blood sugar levels and increase the risk of diabetes.

You will have regular blood tests to check this.

Contact the hospital on the 24-hour number if you have these symptoms during treatment or after it ends:

- feeling thirsty all the time
- needing to pass urine (pee) more often than usual
- feeling more tired than usual.
- losing weight without trying.

If you have diabetes, your blood sugar levels may be higher than usual. You may need advice and support from your GP or diabetes team about managing this.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

[If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Hair loss

Your hair may get thinner. But you are unlikely to lose all the hair from your head. Hair loss usually starts after your first or second treatment. It is almost always temporary, and your hair will usually grow back after treatment ends.

Your nurse can talk to you about ways to cope with [hair loss](#).

Feeling tired (fatigue)

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

Sometimes, this treatment can affect glands that make hormones. This can make you feel tired. Contact the hospital on the 24-hour number if either of these things happen during treatment or after it ends:

- your tiredness gets worse or stops you doing your usual daily activities
- you feel weak or shaky.

If you feel sleepy, do not drive or use machinery.

Hearing changes

Cisplatin may cause hearing changes, including hearing loss. You may have ringing in the ears. This is called tinnitus. You may also become unable to hear some high-pitched sounds. Hearing changes may get better after this treatment ends. But this does not always happen. If you notice any changes

in your hearing, tell your doctor, nurse or pharmacist.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Fluid build-up

This treatment can cause a build-up of fluid in the body. This will slowly get better after treatment ends. Contact the hospital on the 24-hour number if you:

- are gaining weight
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Eyesight changes

This treatment can affect your eyes and eyesight. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during treatment or after it ends:

- dry, itchy, painful, or swollen eyes
- eyes that are sensitive to light
- changes to your eyesight.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Sex

It is possible that small amounts of chemotherapy may be passed on through vaginal fluids or semen. If you have sex in the first few days after treatment, your cancer team will usually advise using condoms or a dental dam to protect your partner.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Date reviewed

Reviewed: 01 January 2025 | Next review: 01 January 2027

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Get in touch



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Our Support Line is available Monday to Friday, 9:30am-4:30pm



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